Question:

A baby is born after the 32nd gestational week by cesarean delivery. The mother suffered from gestational diabetes; however, she had no other pregnancy-related diseases and was otherwise healthy. The baby has a blood pressure of 100/58 mm Hg, heart rate of 104/min, and oxygen saturation of 88%. The child has tachypnea, subcostal and intercostal retractions, nasal flaring, and cyanosis. The cyanosis is responding well to initial administration of oxygen. The nasogastric tube was positioned without problems. Which of the following is the most likely diagnosis?

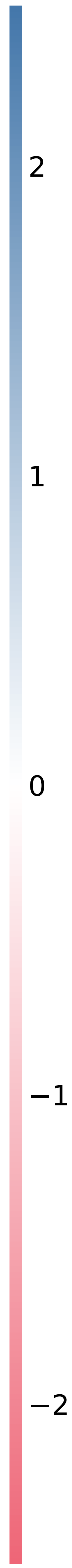
Answer Choices:

A. Neonatal respiratory distress syndrome (NRDS)

B. Tracheoesophageal fistula

C. Pneumonia

D. Congenital heart anomaly with right-to-left shunt



Reasoning + Answer:

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The clinical scenario involves a newborn with tachypnea, retractions, nasal flaring, and cyanosis, all of which are signs of respiratory distress. The cyanosis improves with oxygen administration, and the newborn has normal feeding, implying no issues with gastrointestinal or tracheoesophageal issues.

- NRDS (Neonatal Respiratory Distress Syndrome) typically affects preterm infants and is characterized by surfactant deficiency leading to alveolar collapse. However, this infant is term and had no signs of prematurity, making NRDS less likely.
- Tracheoesophageal fistula would typically present with feeding difficulties and aspiration, and nasogastric tube placement would be problematic. Since these signs are not present, this is unlikely.
- Pneumonia can cause respiratory distress but typically involves symptoms like cough, fever, and consolidation on auscultation. The clinical presentation and absence of these additional signs make pneumonia less likely.
- Congenital heart anomalies with right-to-left shunts can lead to cyanosis without need for oxygen and cause respiratory distress with signs of hypoxia. This fits the clinical presentation well.

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</think>

<answer>D. Congenital heart anomaly with right-to-left shunt</answer>